



Guidelines

Specialty Area Admission Criteria

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

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Introduction

Speciality areas may have specific criteria for admission, discharge and transfer. Refer to the following sections for details:

- OPH Surgical Services - Ward 6
- Intensive Care Unit (ICU) - G44
- Coronary Care Unit (CCU) - G42
- Cardiology Overnight Procedure (COP) Beds - G41
- Short Stay Unit (SSU)
- Surgical Assessment Unit (SAU) - G71
- Neurosurgical High Dependency Unit - G52
- Respiratory Special Care Unit (ReSCU) - G54
- Cardiothoracic Unit - G62
- Endovascular Neurointerventional Unit (ENU) - G66

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☐
Other:	☐		

1.0 OPH Surgical Services - Ward 6

OPH accepts admissions for elective procedures and surgery for same day-stay in the Theatre Suite and for multi-day stay on Ward 6.

Ward 6 has a cohort of 12 beds allocated to acute surgical patients from SCGH. Patient details (including relevant clinical information) are entered onto the Enterprise Bed Management System (EBM) by SCGH.

Elective patients are registered on a certified waiting list. OPH compiles this waitlist from the Waitlist Inclusion Form received from accredited medical practitioners (surgeons and anaesthetists).

Only medical practitioners (surgeons/anaesthetists) who are accredited to the Hospital or a nominated and hospital-accredited locum can admit and treat patients at OPH.

1.1 Medical Criteria for Surgical Patients

Absolute Exclusion Criteria:

- BMI ≥ 45 or weight $> 130\text{kg}$
- Complex chronic pain ($> 60\text{mg}$ daily oxycodone/day) or IV drug use (IVDU)
- Unstable psychiatric disorder or security risk
- Unsuitable discharge arrangements (no fixed abode)
- Prisoners
- Surgically complex operations (bilateral or revision arthroplasty, free flap or complex reconstructive surgery, radical tumour surgery, major upper GI surgery, significant blood loss anticipated).

1.2 Pre-Admission Clinic

Elective surgery patients are referred if the following criteria are met:

- Major surgery
- Patient conditions:
 - Unable to lie flat
 - BMI 40 -45
 - Oral anticoagulants (excluding aspirin)
 - Significant drug allergies or anaphylaxis
 - Poor exercise tolerance
 - Recent clinical deterioration
 - Insulin dependent diabetes
 - History of stroke or recent TIA
 - Potentially difficult airway
 - History of anaesthesia complications
 - Non-English speaking or potential difficulty with consent

1.3 Emergency Surgery

Contact the Speciality Consultant, Duty Anaesthetist, Theatre Area Manager and Ward 6 CNM

2.0 The Intensive Care Unit (ICU) - G44

2.1 General Admission Criteria

- The Intensive Care Unit (ICU – G44) is a 30 bed Level III tertiary referral unit.
- Admission is based on potential benefit and must be approved by the ICU Consultant or Senior Registrar. ED reviews by ICU medical staff should occur as soon as practicable, and a plan of care be documented. Once accepted, notify the ICU Shift Coordinator immediately to arrange timely transfer.

- Patients with documented wishes not to receive treatments normally performed in ICU in an Advanced Health Directive (AHD) or [NMHS Goals of Patient Care Policy \(GOCP\)](#) should not be admitted.

2.2 Admission Categories

Category 1: Critically ill, unstable patients in need of Critical Care level treatment and monitoring, including respiratory and ventilator support, continuous vasoactive drugs, sympathomimetic amine, Continuous renal replacement therapy (CRRT), Extracorporeal Membrane Oxygenation (ECMO), and multiorgan failure.

Category 2: Patients requiring intensive monitoring or have the likelihood of immediate intervention e.g. post cardiothoracic, neurosurgical, thrombolysis or major complex surgery.

Category 3: Patients who are critically ill but have a reduced likelihood of recovery because of underlying disease or the nature of their acute illness. Referring specialists should provide a clear indication of their goals of care when requesting admission.

Category 4: Patients admitted to SCGH as part of a state service area of excellence, such as KEMH support, Neurological Intervention & Imaging Service of Western Australia (NIISWA) and Liver failure.

Category 5 (unsuitable): Patients, who because of their underlying disease or severity of the condition and background health have no chance of benefit from ICU interventions, and for whom such treatments would be inappropriate. These patients should not be admitted to ICU, the exception being those patients who have expressed a desire to become an organ donor.

Patients admitted under Cardiology, Respiratory Medicine, Cardiothoracic surgery and Neurosurgery who are suitable for management in their respective High Dependency Unit.

Factors to be considered when assessing suitability for admission:

- Diagnosis
- Severity of illness
- Goals of Care
- Age and Frailty Score
- Coexisting disease
- Physiological reserve
- Prognosis
- Availability of suitable treatment
- Response to treatment to date
- Recent cardiopulmonary arrest
- Anticipated quality of life
- The patient's wishes and advanced care directives

2.3 Procedure for Admission

All patients are admitted under the relevant medical or surgical team (bed card). Whilst in the ICU, the ICU Consultant on duty is responsible for immediate management and coordinating the care of the patient.

2.3.1 Planned/Elective Admission

Prior to admission

- The surgical team must submit an eReferral prior to 1300 hours on the day prior to surgery.
- Referrals received after 1700 hours must be discussed with the ICU Consultant or their delegated Senior Registrar.
- The ICU Consultant or their Senior Registrar delegate will review cases and accept, reject or suggest review in recovery based on clinical need.

Scheduled day of surgery

- The first two cases on the theatre list are prioritised for ICU beds.
- The Theatre Floor Coordinator will liaise with the ICU Shift Coordinator/CNS to confirm bed availability to proceed with further cases requiring post operative beds in ICU daily and inform duty anaesthetist
- If surgery is cancelled, a referral should be resubmitted, on the day preceding surgery.
- An ICU Consultant will discuss cases not readily covered by the above procedural plan

2.3.2 Complicated Planned Admission.

Cases requiring multi-modality input (specialised surgeon or clinician availability postop), specialised nursing or clinical input (e.g. paediatric cases) or the use of unusual therapies (e.g. specialised chemotherapy or dialysis-pheresis) should be notified to the ICU well in advance of the standard 24-hour notification time. If ICU treatments will be required in theatre (e.g. Nitric or CRRT), ICU must be notified well in advance to ensure availability of nursing staff and equipment to attend theatre.

2.3.3 Non-Elective Admission

- The registrar of the admitting team must be notified, and if possible, review the patient prior to admission.
- Admissions may come from ED, Operating Theatre (SCGH) or other hospitals. The ICU accepts direct admissions from other metropolitan, rural and international regions.
- The ED and ICU will discuss whether patients will be directly admitted, or whether admission will follow stabilisation and investigation outside the ICU.
- Where the ICU has no capacity to accept the admission or where the patient may benefit from admission to another ICU, the ICU Consultant on duty or their delegate will negotiate with other ICU to identify a suitable placement.

Request for admission

- Direct to the ICU Senior Registrar (SR) or Consultant on duty who will assess the request for admission and inform the ICU shift coordinator of the patient's clinical condition prior to transfer.
- Requests for patient review should only occur with the knowledge of the admitting team Consultant or their delegate.

Patients not accepted for admission

- The ICU reviewing doctor will provide appropriate documentation in the DMR for those patients who are deemed not suitable for ICU admission.

Ward patients reviewed by ICU

- Where ICU admission follows a requested ward review, the ICU reviewing doctor will document a summary of admission arrangements in the DMR.

2.4 Discharge from ICU

- Patients are discharged when ICU level care is no longer needed.
- The decision will be made by the ICU duty Consultant or SR.
- Patients will be discharged to the care of the specialty team as specified by the bed card (generally the admitting specialty).
- Where the specialty team (as specified by the bed card) feel that an alternate specialty is more appropriate for the patient to be discharged to, the speciality team must ensure that involvement and handover to that specialty occurs professionally and without risk to the patient. This should not inappropriately delay discharge from the ICU.

2.5 Procedure for transfer to the ard

- ICU Shift Coordinator enters patient details (including relevant clinical information) into the Enterprise Bed Management System (EBM).
- Transfers are discussed at the 0900 & 1345 hospital bed meetings.
- Ultimately the decision to admit a patient to the ICU and to decide when they are medically suitable to be discharged is the responsibility of the duty ICU Consultant or SR and is made in collaboration with the admitting team.

3.0 Coronary Care Unit (CCU) - G42**CCU accepts admissions from the following sources:**

- SCGH Emergency Department (ED).
- Code STEMI from JHC and SJA.
- Transfers from Cardiology - G41 - or general wards.
- Inter-hospital transfers
- Direct admission of elective patients and/or planned or unplanned Cardiovascular Invasive Laboratory cases.

3.1 SCGH ED Admissions

- ED medical staff commence initial assessment and management
- Refer to the CCU Registrar
- CCU Registrar assesses within 30 mins of referral
- CCU Registrar will advise the CCU Shift Co-ordinator of patient details, clinical assessment and initial management plan
- The CCU shift co-ordinator enters bed / time ready details into EBM.

3.2 CODE STEMI Admissions:

- May be accepted for transfer by the on-call Registrar or Consultant *before* notifying CCU or Bed Manager. The medical officer accepting the patient will notify the CCU shift co-ordinator. The patient may arrive before the bed is available.
- If no CCU bed is:
- Transfer a stable CCU patient to G41
- Admit to ICU
- Hold in CVIL recovery until CCU bed is ready (this is to be in consultation with the CVIL nursing staff)

- Consider transferring patients from G41 to SSU if cardiac monitoring is not required.
 - If bed availability is an issue, the on-call Cardiologist is to be informed.

3.3 CCU (G42) & G41 Admissions:

- CCU and G41 operate as a single unit for the purposes of bed management.
- The CCU/G41 CNS will identify patients to be transferred to and from CCU and G41 before the 0900hrs & 1345hrs Bed Meeting and informs the Bed Manager.
- CCU shift coordinator *only accepts* CCU admissions from the CCU or on call Cardiology Registrar.
- If an ED patient is deemed suitable for admission to G41 and no bed is available, the patient may be admitted to CCU if approved by the on-call cardiologist.
- Patients under other specialties can only be admitted to G41 for cardiac monitoring following consultation and acceptance by the Cardiology team. This patient will be known as Shared Care, between the referring specialty and Cardiology.

3.4 Transfer of patients out of CCU and G41 (out-lie) procedure:

- At 1500hrs on weekdays, the CCU/G41 CNS and CCU and G41 S/C will identify suitable patients to out lie.
- After hours the decision will be made by the on-call Cardiology Registrar in consultation with the Shift Coordinator.
- G41 Cardiology patients that are suitable to out lie include:
 - awaiting CABG and/or Valve Replacement surgery within 1-2 days. These should be transferred to G62.
- G41 Cardiology patients not suitable to be out-lie include:
 - Patients on either hard-wire or telemetry cardiac monitoring
 - Patients with behavioural or cognitive issues.

4.0 Cardiology Overnight Procedure (COP) Beds - G41

Hours of Operation: Monday 0600 to Saturday 1200

For elective Cardiology patients before and after their procedure in the CVIL requiring monitoring overnight.

- All patients are admitted to G41 under a cardiovascular medicine team
- Expected hospital stay is <23-hour
- Unoccupied COP beds may be used for:
 - cardiology admissions
 - transfers from CCU.

5.0 Short Stay Unit (SSU)

- All patients are admitted under a specialty team responsible for their management during their stay in the SSU.
- SSU beds are primarily allocated to patients undergoing elective procedures or surgery.
- Unoccupied SSU beds may be considered for
 - elective medical patients requiring overnight admission
 - emergency surgical patients requiring overnight admission

The Short Stay Unit comprises two main areas:

1. **Day Case Admission (DCA):** for patients post procedure or surgery and are discharged home on the same day.
2. **Day of Surgery 23-Hours Unit (DO23):** for patients requiring a 1- or 2-night hospital stay post elective surgery or an elective procedure. Patients are admitted on the day of their surgery/procedure, recover in the Unit and are discharged once deemed medically fit.

5.1 Day Case Admission (DCA)

Hours of Operation: Monday to Friday 0700 – 2130 hours.

5.1.1 Patient Selection

- Patients requiring a surgical or medical procedure that does not require overnight admission.
- ASA 1 or ASA 2 (relatively fit patients with a stable coexisting condition). Patients outside of these parameters may be admitted **only** at the discretion of the Clinical Nurse Specialist.
- Patients having general anaesthesia or sedation must have a responsible adult carer available to provide transport and care for them in the first 24 hours post-surgery.

5.1.2 Pre-admission assessment

- **General Anaesthetic:** all patients must complete a health questionnaire upon being waitlisted for surgery.
- **Patients ≤ 60 years:** Not all patients require pre-admission assessment The need is determined by the health questionnaire which is reviewed by Pre-Admission Clinic nursing staff.
- **Patients > 60 years:** All must attend a PAC pre-admission assessment. The type of assessment (phone or clinic) is determined by the health questionnaire.
 - Patients who do not return the health questionnaire will be scheduled for a face-to-face assessment with the PAC multidisciplinary team.
 - Patients undergoing procedures under local anaesthetic do not require preadmission assessment.

5.1.3 Post procedure Care

- Minimum post-anaesthetic length of stay is two (2) hours.
- If post-operative surgical or medical review is required to determine the patient's fitness for discharge this must occur before 18:00. All further reviews must be completed by 20:00.
- Patients are not permitted to drive or travel unaccompanied on public transport post general anaesthetic or sedation.
- Patients must be collected and cared for by a responsible adult for the first 24 hours post general anaesthetic or sedation.
- Patients who do not meet discharge criteria will be transferred to an overnight bed under their admitting team.
- Patients requiring admission will be admitted under the consultant who performed the procedure or surgery.

5.2 Day of Surgery 23 Hour Unit (DO23)

Hours of operation: Monday 1100 – Saturday 1530 hours

DO23 is co-located with DCA and has the capacity for 22 elective surgical patients

5.2.1 Patient selection

- Surgical patients expected to have a length of stay (LOS) of **48 hours or less**
- Patients must be suitable for care in a mixed gender, ambulatory care-style facility.
- Patients must be medically stable, have minimal co-morbidities, be reasonably independent, compliant and orientated.

5.2.2 Exclusion Criteria

- Patients requiring single room accommodation due to infection control risks.
- Patients with high-risk airway management requirements.
- Patients requiring bowel preparation (due to limited bathroom facilities).
- Patients who are aggressive, have altered mental state, or are likely to wander Patients requiring multiple day stay with no definite plan of care.

5.2.3 Medical guidelines

- All patients must be reviewed by the surgical or medical team prior to 09:30 to enable discharge by 10:00.

6.0 Surgical Assessment Unit (SAU) – G71

All admissions to the SAU should have confirmation of a probable surgical condition.

6.1 Inclusion Criteria

- Major Trauma
- Acute abdomen with signs of local or generalised peritonitis including pancreatitis
- Intestinal obstruction
- Rectal & or lower GI bleeding
- Abdominal wall pathology complicated hernia, stabbing and trauma
- Soft tissue infections of the torso (including perianal infections)
- Complications of abdominal surgery performed at SCGH or referred for tertiary care
- Breast surgery complications
- Emergency complications of Bariatric Surgery
- Obstructive cholangitis, appendicitis

6.2 Exclusion Criteria

- Haematemesis -the General Surgical team should be informed if the patient is hemodynamically unstable on arrival in ED
- Vomiting and diarrhoea
 - Exceptions: Bloody and/or diarrhoea and hemodynamic instability
- Cellulitis without a drainable collection & cellulitis of the limbs
- Biliary colic resolving with adequate analgesia
- Patients with a hepatic LFT profile.
- Constipation without aperient in ED
- Head Injury patients with neurological signs following head trauma.
- Spinal Injury requiring Cardiac Monitoring

7.0 Neurosurgical High Dependency Unit – G52

Is a 9-bed unit with the potential to expand to 11 beds for post operative cranial neurosurgical patients requiring a high level of observation and intervention. Contains two single rooms and 7 open bed areas.

7.1 Referrals

Emergency Department (ED) referrals will require Registrar to Registrar discussion and timely review by the Registrar on call following discussion with the Neurosurgical Consultant. Direct referrals from ICU and theatre will be accepted if under a Neurosurgical bed card and meet the inclusion criteria.

7.2 Inclusion Criteria

Patients suitable for G52 HDU include:

- Post intracranial surgery.
- Acute aneurysmal subarachnoid haemorrhage, or acute arterial-venous malformation haemorrhage.
- Acute traumatic brain injury and patients who have had either a subdural, intraparenchymal or extradural haemorrhage evacuated
- External ventricular drain (EVD) and or Intracranial pressure (ICP) monitoring
- Endovascular treatment such as coiling of an aneurysm or stenting through NIISWA (emergent or elective) treating a neurosurgical issue
- Need for close monitoring (inclusive of arterial monitoring) or full neurological observations performed at hourly or more frequent intervals.
- Invasive EEG monitoring for the first 24 hours post operatively.

7.3 Exclusion Criteria

Not suitable for G52 HDU:

- Patients requiring telemetry for rhythm analysis and assessment
- 1:1 care for multi organ failure, risk of respiratory failure or airway risk (unsecured airway)
- Inotropic support
- Patients requiring observation by HSA or security surveillance not meeting any of the above inclusion criteria for HDU.
- Airborne infection precautions' (excluding COVID 19) in accordance with the infection prevention and control guidelines
- Neurology patients requiring close observation will be managed on Ward G51
- Post thrombectomy neurology patients will be managed in ICU
- Neurosurgical patients managed on Ward G66: -
 - Elective or emergency spinal surgery (all levels) including patients with chest drains following thoracotomy
 - Insertion of a Deep brain stimulator

7.4 Discharge Criteria

- Neurologically stable with stable vital signs and medically cleared
- No longer require invasive monitoring – Arterial lines and EVD's must be removed prior to transfer to ward area.
- The decision to discharge may be a consultative process between medical and nursing

8.0 Respiratory Special Care Unit (ReSCU) – G54

Is a dedicated six-bed, hospital-wide service within the Division of Cancer, Imaging and Clinical Services, managed by the Department of Respiratory and Pulmonary Physiology.

The Unit provides an intermediate level of care for patients with acute or complex respiratory failure who require advanced oxygen therapy, non-invasive ventilation (NIV), or close respiratory monitoring that exceeds the capability of a general medical ward but does not meet criteria for Intensive Care (ICU-G44). ReSCU functions as a high-acuity respiratory observation and management area, supporting safe, cost-effective care, timely escalation of deteriorating patients, and step-down from ICU where appropriate. All admissions require discussion and approval by the on-call Respiratory or Pulmonary Physiology Consultant. The key nursing contact is the Clinical Nurse Specialist (CNS), Respiratory Medicine, ward G54.

8.1 Admission Criteria

Patients may be admitted for:

- Type 1 hypoxaemic respiratory failure requiring high-flow oxygen or high-flow nasal oxygen (HFO/AIRVO).
- Type 2 hypercapnic respiratory failure requiring NIV or close monitoring, including regular arterial/venous blood-gas assessment.
- Commencement or titration of complex home NIV therapy, including initiation of long-term ventilation programs.
- Ventilator weaning following acute respiratory failure, including via tracheostomy, where invasive ventilation is no longer required.
- High-risk or intermediate-risk pulmonary embolism (as per [PE algorithm](#)) requiring close monitoring and potential respiratory support.
- Patients with neuromuscular disease (e.g., motor neurone disease, muscular dystrophy) or obesity hypoventilation syndrome requiring ventilatory support.
- Post-procedural or peri-operative respiratory patients with significant respiratory comorbidities (e.g., bronchoscopy, thoracic procedures) requiring enhanced observation.

Patients under other specialties can be admitted for respiratory support and enhanced monitoring following consultation and acceptance by the Respiratory or Pulmonary Physiology team to ensure appropriate medical governance and review. This patient will be known as Shared Care; between the referring specialty and Respiratory, unless explicitly taken over care (TOC) by the Respiratory team.

8.2 Exclusion Criteria

ReSCU is not suitable for patients who:

- Require invasive mechanical ventilation or continuous NIV (> 30 minutes intolerance off device).
- Require cardiac rhythm monitoring beyond simple 3-lead ECG or rate monitoring (patients needing telemetry should be managed in CCU).
- Have multi-organ failure, altered consciousness compromising airway protection, or require continuous intravenous sedation.
- Require isolation precautions needing a negative-pressure or single-room environment (unless pre-approved with appropriate staffing plan).
- Require complex non-respiratory critical care outside the respiratory domain (refer to ICU-G44).
- Oxygen requirements >0.5 FiO₂ without goals of care or escalation plan in place
- Conditions outside the usual indications for NIV (e.g., pneumonia and asthma)
- Pulmonary oedema requiring CPAP or NIV – these should be discussed with the CCU for admission.
- Asthma requiring intravenous salbutamol or aminophylline infusions (should be nursed in ICU)
- Respiratory failure from opiate overdose requiring a naloxone infusion.

- Fixed upper airway obstruction.

8.3 Nursing and Staffing Model

- Standard nurse-to-patient ratio: 1:2-1:3.
- Escalation to 1:1 based on clinical acuity and in consultation with the CNS or Nurse-in-Charge and the admitting Consultant.
- All nursing staff are trained in the management of NIV, HFNO/AIRVO, and acute respiratory monitoring.
- Allied Health input includes physiotherapy, dietetics, speech pathology, and social work as required.

8.4 Escalation and referral to ICU

ReSCU operates within the A-ORC escalation pathway. There is no unit medical cover, this is provided by the admitting medical team and standard after hours medical staff. The Escalation to ICU-G44 must be promptly considered for patients who:

- Require $\text{FiO}_2 > 0.5$ or rapidly escalating oxygen therapy, if in line with care goals.
- Require continuous NIV or invasive ventilation.
- Develop multi-organ dysfunction or haemodynamic instability.
- Have deteriorating gas exchange or consciousness despite maximal ReSCU support.

Early communication with ICU and activation of escalation pathways (e.g. MET, Sepsis) is mandatory to ensure safe transfer and continuity of care.

8.5 Discharge / Step-down Criteria

Patients may be discharged from ReSCU when:

- Ventilatory or oxygen requirements reduced to general ward level.
- No longer requires continuous monitoring or frequent blood-gas assessment.
- Have achieved a stable airway and respiratory pattern with agreed goals of care.
- The decision to discharge may be a consultative process between medical and nursing team as clinically indicated.

9.0 Cardiothoracic Unit - G62

Is a 4-bed unit specifically equipped and staffed to provide post-operative care to cardiac surgery patients. Most patients will 'step-down' to the unit from ICU and as such no specific referral is required. Admission to the unit is determined through collaboration between Cardiothoracic Registrars and G62 CNS.

The length of stay within this unit is at least 24 hours for arrhythmia monitoring and is influenced by inpatient surgical demand and flow of cardiac surgery patients from ICU. Patients progress to general ward beds where ongoing cardiac monitoring can be via telemetry.

Beds within this unit are utilised as general ward beds if there are no cardiothoracic patients requiring admission to the unit.

9.1 Inclusion Criteria

Patients suitable for G62 include those recovering from:

- Open or minimally invasive cardiac surgery for arrhythmia monitoring and management of epicardial pacing
- Atrial ablation, pulmonary vein isolation and left atrial appendage occlusion
- Pneumonectomy
- Post thoracic surgery if cardiac monitoring required

- Postoperative complications such as tamponade, pneumothorax or atrial arrhythmias

9.2 Exclusion Criteria

Not suitable for G62:

- Patients requiring inotropes or arterial monitoring
- Patients requiring 1:1 care for non-invasive ventilation
- Patients requiring cardiac ST segment monitoring for myocardial ischaemia
- Patients requiring constant observation by AIN companion or security surveillance not meeting any of the above inclusion criteria.
- Patients who require a single isolation room in accordance with the infection prevention and management guidelines

10.0 Endovascular Neurointerventional Unit (ENU) – G66

Is a 2 bedded unit specifically equipped and staffed to care for patients returning from NIISwa after treatment of unruptured aneurysms, as well as a small number of other patients as outlined in the inclusion criteria below. Each bed space is fully monitored, with a nurse-to-patient ratio of 1:2.

10.1 Referrals

Only planned admissions - such as DOPA and DOSA patients - will be accepted into the unit. These admissions are coordinated by the NIISwa and Neurosurgical teams in consultation with the G66 Clinical Nurse Specialist (CNS).

10.2 Inclusion Criteria

Patients eligible for admission to G66 ENU include:

- Elective endovascular procedures via NIISwa, such as coiling of an unruptured aneurysms or venous stenting.
- Spinal neurosurgical patients who require close monitoring, including arterial line monitoring or full neurological observations at hourly or more frequent intervals.
- Spinal neurosurgical patients with severe obstructive sleep apnoea requiring close observation.

10.3 Exclusion Criteria

The following patients are not suitable for admission to G66 ENU:

- Carotid stenting.
- Post intracranial surgery.
- Acute aneurysmal subarachnoid haemorrhage or acute arterial-venous malformation haemorrhage.
- Acute traumatic brain injury or those who have undergone evacuation of subdural or extradural haemorrhages.
- Patients with an external ventricular drain (EVD) and/or Intracranial pressure (ICP) monitoring.
- Invasive EEG monitoring for the first 24 hours post operatively.
- Patients requiring 1:1 care due to multi organ failure, risk of respiratory failure or airway risk (unsecured airway).
- Inotropic support.
- Cardiac monitoring.
- Patients requiring observation by AIN or security surveillance who do not meet any the inclusion criteria for HDU.
- Airborne infection isolation room in accordance with the infection prevention and control guidelines.

- Neurology patients requiring close observation will be managed on G51.
- Post thrombectomy neurology patients will be managed in ICU.

10.4 Discharge Criteria

Patients may be discharged from G66 ENU when:

- Neurologically/neurovascularly stable with stable vital signs.
- No longer need invasive monitoring– Arterial lines must be removed prior to transfer to ward area.
- Discharge decision is made collaboratively between medical and nursing.

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